Staff answers and teaching notes

Outcome

Understand mental health and where to get support.

Meaning and limits

Everyone has mental health. It is part of health. It links to thoughts, feelings and how we cope with daily life. Mental health can change. Having mental health does not mean having an illness. A person may need support if they feel low or worried for a while, sleep very differently, stop doing things they like or find daily tasks hard. Signs can have many causes. One sign cannot tell us a diagnosis. A person can ask for help at any time, even with no clear sign.

Acceptable support types

A trusted adult: Listens and helps find a safe next step.

School staff: Can help at school and find more support.

A family doctor: Can discuss health and help find suitable care.

A helpline: Can listen or give advice. Ask staff to help find one suited to the person's age and need.

How to ask

Ask a trusted adult in private. Use words, a note, HELP card or usual aid. If one adult is busy or the help is not enough, ask another trusted adult. School staff can help find a family doctor or a suitable helpline. Do not wait for a problem to be severe.

Misconceptions

Everyone has mental health, including people without a condition. Quietness, one poor night or a single behaviour cannot establish a diagnosis. Support can be sought without a label and before a problem becomes severe. Friends can be kind, but are not responsible for diagnosis, secrecy or solving another person’s concern.

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Fictional answers and checks

A A changed routine

Worry and focus are making a daily task harder. School staff could listen and help plan one clear first step. They can help find more support if needed. Opener: Can we talk in private? I need help with the new routine. If the adult is unavailable or help is not enough, use another trusted adult or the school help route. Other fitting answers with reasons are valid.

B Missing an activity

Feeling low for a while and missing an activity may mean support is needed. A trusted adult could listen and help the person speak to school staff or a family doctor. Opener: Can I talk to you? I have felt low for a while and would like help. If the adult is unavailable or help is not enough, use another trusted adult or the school help route. Other fitting answers with reasons are valid.

C A tired start

A change in sleep and hard daily tasks may mean support is needed. School staff could listen and help the person find a family doctor or a suitable helpline with adult support. Opener: Can we talk in private? I am finding daily tasks hard and need help. If the adult is unavailable or help is not enough, use another trusted adult or the school help route. Other fitting answers with reasons are valid.

Main hinge

B. A sign may mean support would help. A diagnoses; C denies universal mental health; D delays help. Do not diagnose story people. If unclear, re-model and use the parallel.

Parallel hinge

A. Ask a trusted adult privately. B delays and creates secrecy; C removes privacy; D diagnoses from a clue. A pass is unobserved. Read or point privately in usual mode.

Task choices

Map B and note C require the same elements. Supported choices are content prompts if they supply the answer. Normal text reading, aid, typing and exact scribing are access. Aspire compares support types and explains the diagnostic limit without asking for personal disclosure.

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Evidence and safe delivery

Assessment

Record a universal mental-health explanation, a story clue, appropriate support with reason, private opener and fallback separately. Retain partial evidence and actual prompts. No marks for personal health, willingness to disclose or public speech. The learning self-check is not a health screen.

TA focus

Model A, guide a short match or advice repair, then check each chosen task privately. Record actual aids and answer cues. A pass remains unobserved. Do not infer health from the response.

Feedback and banking

Keep the first response. Give one specific NS. The pupil replies in the same colour, on the same page or usual mode. E records actual saved work. R follows only when Voice and Audience are present with one NS and the pupil's actual response. Use work codes, never names. Offer to the Communication portfolio; staff confirm any mapping.

Private card

The one-line personal card is optional and stays with the pupil. Do not collect, photograph, display, compare or copy it into evidence. Any space may stay blank. A fictional response or pass is valid. No adult name, health detail or real story is required.

Safeguarding

Any disclosure goes to the safeguarding lead through school procedure. Listen calmly, reassure and explain that staff must share a concern with people who can help. Do not probe, investigate or promise secrecy. Use the school's immediate-concern route when needed. Keep real details in the school's safeguarding record, separate from learning evidence. A pupil unable to identify a trusted adult is supported privately, not pressed.

Step out

Before the lesson agree a discreet HELP or PAUSE signal. Staff acknowledge it quietly and use the familiar supervised place and agreed adult support. No public explanation is needed. Offer a neutral task, private catch-up and supported return. Do not send a pupil away alone.

Source and scope

Exact GROW Weekly - Spring C66 outcome verified from the source workbook. Owner supplies Wednesday 20 January 2027 P3, up to five learners and forty minutes. Accepted GROW Weeks 1–2 supply structure and access conventions. Current official secondary mental-wellbeing guidance and public-health guidance on young people's signs and support were checked. All cases, tasks and paraphrases are original. No source passages, named service or website reproduced.

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